

 Clinical Diagnostic Summary Report

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REPORT

Automated Deep Convolutional Tri-Ensemble Analysis

Patient / Case ID: CT-AD-1092
Scan Specimen: CT-AD-1092
(Adenocarcinoma Nodule)
Acquisition Matrix: 350 × 350 px (Bilinear)

Attending Clinician: Dr. S. Mukherjee, MD
Department: Pulmonary Diagnostic Center
Medical License: RAD-8820-ONC

CONSENSUS PRIMARY FINDING

Adenocarcinoma

Diagnostic Confidence: 91.26%

Subtype & Anatomical Site: Non-Small Cell Lung Carcinoma (NSCLC) (Peripheral Bronchial Glands / Outer Margins)

Radiological Hallmarks: Ground-glass opacities (GGO), irregular spiculation, subsolid pleural retraction.

Multi-Class Probability Distribution

Adenocarcinoma:	91.26%	Large Cell Carcinoma:	1.26%
Normal (Healthy Lung):	0.00%	Squamous Cell Carcinoma:	7.48%

Tri-Ensemble Consensus Weights

- DenseNet121 + CLAHE: 60% weight (Concatenated low-level edge & contrast preservation)
- EfficientNetV2-S: 30% weight (Fused-MBConv multi-scale receptive field analysis)
- Xception: 10% weight (Depthwise separable spatial gradients & micro-spiculation)
- 5-View Test-Time Augmentation (TTA) consensus applied

Safety Protocol Verification: Patient scan evaluated under 99.62% Malignancy Recall safety threshold with 0 missed cancers recorded on holdout validation.

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